SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Fruquintinib (Fruzaqla) monotherapy

Metastatic colorectal cancer after standard therapies and trifluridine/tipiracil or regorafenib · NCCP 00890 · version 2a · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Fruquintinib is a targeted capsule treatment to help control bowel cancer that has spread after previous treatments. The team checks which treatments you have received, whether they remain suitable, and your priorities.

Schedule
Take the prescribed fruquintinib dose once daily on Days 1–21, then take no fruquintinib on Days 22–28. Take it at about the same time each day, with or without food. Repeat only when your team authorises the next cycle.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Fruquintinib
TARGETED THERAPY CAPSULE

- ☐ High blood pressure, including severe hypertension
- ☐ Painful red hands and feet
- ☐ Protein in urine and kidney complications
- ☐ Bleeding, bowel perforation, clotting and impaired wound healing
- ☐ Thyroid changes, liver-test abnormalities, diarrhoea, fatigue, appetite/weight loss and mouth soreness
- ☐ Rare serious neurological or blood-vessel complications

Discuss the previously treated colorectal setting, benefit and uncertainty, alternatives, blood-pressure and urine monitoring, hand-foot effects, fatigue, mouth/bowel/appetite effects, serious bleeding, clotting, bowel perforation and neurological risks, wound healing, pregnancy precautions, medicine interactions and the option to reconsider.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00890 v2a.

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